INDIA HEALTHCARE ACQUISITION

# Sector Scan and Pre-Commitment Validation Framework

Prepared for the Executive Committee | 31 October 2026

## 1. Bottom line

The sector offers real structural opportunity, but India healthcare risk is concentrated in a small number of target-specific issues that can only be resolved with the target's own documents. This scan does not assert that the target is attractive or unattractive; it identifies what must be true for approval, and which findings would stop or reprice the deal.

**We recommend that approval be made conditional on eight validation gates (Section 4).** Four are potential deal-breakers independent of price: licence validity and transferability (Q3), the transaction approval path (Q4), the lawfulness of the target's data practices under the DPDP Act (Q7), and unresolved patient-safety or product-quality failures (Q8). Financial quality (Q9) and AI claims (Q6) are the most likely drivers of a price adjustment.

### How to read this document

- The target's subsector (pharmaceutical, device, hospital / care services, digital health) has not been specified. The ten questions therefore apply across subsectors, and each notes where the evidence differs. Once the target is confirmed, Q1, Q3, Q5, Q8 and Q9 should be tailored first.
- This document contains no market sizes, growth rates, multiples or company-specific claims, deliberately. Any figure used in the investment case must be sourced from named, dated evidence during diligence. Where this document refers to laws, they reflect our understanding at the time of writing; counsel must confirm the current text, notifications and commencement dates.
- DPDP Rules, 2025 are addressed in Question 7 and Section 5. Our engagement data governance is in Section 6; it is provisional because the PwC internal policy has not been provided.
- Each question states why it matters, the evidence needed, whether it is sector-level or target-specific, and what it may reveal (opportunity, consolidation, threat, deal-breaker).

## 2. Question map

| # | Strategic question (short form) | Research area | Primary level |
|---|---|---|---|
| **Q1** | Is target growth tied to durable demand drivers it can capture? | 1. Market and demand | Sector + target |
| **Q2** | Where does the target sit in consolidation, and what does precedent support on price? | 2. Competition, M&A / PE | Sector + target |
| **Q3** | Are licences valid, transferable and is enforcement history clean? | 3. Regulatory | Target |
| **Q4** | How exposed are margins to price controls, and is the approval path clear? | 3 and 7. Regulatory / financial | Sector + target |
| **Q5** | How mature and integrable is the technology estate? | 4. Technology | Target |
| **Q6** | Is the AI real, safe, governed and lawfully built? | 5. AI governance | Target |
| **Q7** | Is personal data handled lawfully under the DPDP Act, and how secure is it? | 5. Data, privacy, cyber | Target |

| # | Strategic question (short form) | Research area | Primary level |
|---|---|---|---|
| **Q8** | Can quality, supply and staffing be sustained? | 6. Operations | Target |
| **Q9** | How good is the revenue, and what valuation does it support? | 7. Financial | Target |
| **Q10** | What could change the economics, and does the case survive downside scenarios? | 8. Emerging threats | Sector + target |

## 3. The ten strategic questions

**Q1 | Area 1: Market size, growth, demand drivers**

**Which healthcare subsector and geographic pockets of India are driving durable demand growth, and is the target's growth tied to those structural drivers or to a narrow, cyclical or subsidy-dependent niche?**

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| **Why it matters to the investor** | The growth case underpins the valuation multiple. India is not one market: hospital demand in metro and Tier-2/3 cities, pharmaceutical demand (domestic branded generics versus exports and CDMO), device demand (import-dependent categories versus local manufacturing) and digital health demand behave very differently. A premium paid for "India growth" is unsupported unless the target's specific segment and catchment actually capture it. |
| **Evidence / metrics needed** | • Segment-level market sizing and growth from named, citable sources (Ministry of Health and Family Welfare / National Health Authority data, NSO surveys, IQVIA or equivalent pharma audit data, CDSCO and customs data for devices, industry bodies such as NATHEALTH, AdvaMed India, IPA, and recognised advisory reports). Record source, date and methodology for every figure.<br>• Target's revenue by subsegment, city tier, state, product / service line and customer type for at least the last three years; same-store or like-for-like growth separated from acquired or new-capacity growth.<br>• Catchment analysis: demand, competitor density and payer mix around each facility, plant or sales territory.<br>• Evidence of demand drivers (insurance penetration, public scheme enrolment, chronic disease burden, medical value travel, ageing) linked to the target's actual volumes, not just to the sector. |
| **Sector vs target** | Both. Sector-level for the demand drivers; target-specific for whether the target's mix, geography and capacity capture them. |
| **Signal it may reveal** | Opportunity: under-served Tier-2/3 geographies or adjacent subsegments for bolt-ons. Deal-breaker risk: growth concentrated in one state, one payer scheme, or one product, or growth that is price-led rather than volume-led. |

**Q2 | Area 2: Competitive landscape, M&A and PE activity**

**Where does the target sit in the consolidation of its subsector, who are its real competitors and likely rival bidders, and what do precedent transactions imply for a defensible entry valuation and for exit options?**

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| **Why it matters to the investor** | Competitive intensity determines pricing power and margin durability. Active PE and strategic consolidation can support valuation but also inflate entry prices and create exit-multiple risk if sentiment turns. Rival bidders affect negotiating position, and consolidation by larger platforms can erode the target's standalone economics. |

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| **Evidence / metrics needed** | • Precedent transaction set (deal value, EV/Revenue, EV/EBITDA where disclosed, structure, earn-outs) from stock exchange disclosures, MCA filings, Competition Commission of India (CCI) combination notices and orders, and commercial deal databases. Flag where multiples are not disclosed rather than estimating them.<br>• Listed-peer trading multiples and operating benchmarks (ARPOB and occupancy for hospitals; gross margin, field force productivity and chronic-segment share for pharma; installed base and service revenue for devices; retention and unit economics for digital health).<br>• Market share by segment and geography; competitor capacity additions announced; recent PE entries and exits in the same subsector.<br>• CCI thresholds, including the deal-value threshold introduced by the 2023 amendment, to assess whether the transaction needs notification and whether competition overlap invites remedies. |
| **Sector vs target** | Sector-level for consolidation trend and investor appetite; target-specific for market share, local overlap and bidder dynamics. |
| **Signal it may reveal** | Consolidation opportunity (platform plus bolt-ons), competitive threat from better-capitalised platforms, and a possible deal-breaker if the target's price is anchored on multiples that precedent deals do not support. |

| Q3 \| Area 3: Regulatory environment and compliance<br>**Does the target hold every licence, registration and accreditation it needs, are they valid and transferable on a change of control, and does its enforcement and inspection history show systemic compliance weaknesses?** | |
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| **Why it matters to the investor** | In Indian healthcare, the licence is often the asset. A lapsed, mis-scoped or non-transferable approval can halt production, sales or patient services. Enforcement history is the best predictor of future remediation cost, supply interruption and reputational damage, and of liabilities that survive a share purchase. |
| **Evidence / metrics needed** | • Licence register mapped to activity, site and product: manufacturing and sale licences under the Drugs and Cosmetics Act, 1940 and Rules; Medical Devices Rules, 2017 registrations and licences (CDSCO); clinical establishment registrations under the Clinical Establishments Act, 2010 or state equivalents; NABH / NABL accreditations; pharmacy, radiation (AERB), biomedical waste and fire / building approvals; medical council registrations for clinicians; telemedicine compliance where relevant.<br>• Regulator inspection reports, show-cause notices, "not of standard quality" alerts, product recalls, licence suspensions, warning letters or inspection observations from foreign regulators if the target exports (for example USFDA, EU, WHO-GMP), and the status of closure of each.<br>• Compliance with the revised GMP requirements (Schedule M) for manufacturers, and with the Uniform Code for Pharmaceutical Marketing Practices where applicable.<br>• Legal opinion on whether each licence continues, needs intimation, or needs fresh approval on change of shareholding or control. Do not assume continuity; confirm authority by authority.<br>• Litigation register: consumer forums, medical negligence claims, criminal complaints, tax and customs disputes, regulatory appeals. |
| **Sector vs target** | Regulatory regime is sector-level; licence validity, transferability and enforcement history are target-specific. |

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| **Signal it may reveal** | Frequently a deal-breaker or price-adjustment trigger. Convert findings into conditions precedent, specific indemnities, escrow or R&W insurance exclusions. |

| Q4 \| Area 3 and 7: Price regulation and transaction approvals<br>**How exposed are the target's margins to price controls and trade-margin or reimbursement caps, and what foreign investment, competition and listing approvals are required for the acquirer to complete and then fund growth?** | |
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| **Why it matters to the investor** | Price-controlled products, device price caps, public-scheme package rates and trade-margin rationalisation can compress margins independent of operating performance. Separately, a foreign acquirer faces entry-route constraints (foreign investment limits and approval routes differ by subsector and between greenfield and brownfield investment), which can alter structure, timing and certainty of closing. |
| **Evidence / metrics needed** | • Share of target revenue and gross profit from products or services under National Pharmaceutical Pricing Authority (DPCO 2013) ceilings, device price controls, CGHS / PM-JAY / state scheme rates, and the history of price revisions.<br>• Current consolidated FDI policy and FEMA pricing rules for the target's activity (automatic versus government route, any caps or conditions), Press Note 3 (2020) considerations if any investor or financing source is linked to a land-border country, CCI approval, and SEBI Takeover Code / listing obligations if the target is listed. Obtain written advice from Indian counsel; policy changes frequently.<br>• Sensitivity of EBITDA to a defined price or margin shock (to be specified by the deal team, not assumed here). |
| **Sector vs target** | Policy is sector-level; revenue exposure and structuring consequences are target-specific. |
| **Signal it may reveal** | Possible deal-breaker if approvals are uncertain or the permitted structure undermines control; otherwise a structuring and valuation lever (staged acquisition, earn-out, conditions precedent). |

| Q5 \| Area 4: Technology adoption and digital transformation<br>**How mature, integrated and scalable is the target's technology estate (clinical, operational and commercial systems), does it interoperate with India's digital health infrastructure, and what would integration with the acquirer's platforms cost?** | |
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| **Why it matters to the investor** | Synergy cases often assume shared platforms, centralised procurement and data-driven productivity. Fragmented or legacy health IT, un-integrated acquisitions already on the target's books, and vendor lock-in raise integration cost and delay. Interoperability and data-quality gaps also limit how much of the target's data is actually usable, including for any AI use. |
| **Evidence / metrics needed** | • Application and infrastructure inventory (EMR / HIS, LIS, RIS / PACS, ERP, supply chain and distributor management, CRM / field force tools, patient apps), versions, hosting location, ownership of source code and key vendor contracts including change-of-control clauses.<br>• Degree of adoption by clinicians and staff (usage data, not licence counts); downtime and incident logs.<br>• Status against the Ayushman Bharat Digital Mission (ABDM) standards (for example health ID / ABHA linkage, health information exchange, facility and professional registries), as applicable to the target. |

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| | • Independent technical due diligence on architecture, technical debt and integration effort; capex and opex run-rate for technology. |
| **Sector vs target** | Target-specific, with sector context on public digital infrastructure and talent availability. |
| **Signal it may reveal** | Integration cost and timeline risk, plus consolidation opportunity if the target's data and workflows can be standardised across a larger platform. |

| Q6 \| Area 5: AI adoption, AI governance and clinical safety<br>**Where does the target actually use AI, is it a genuine, defensible capability or marketing, and is there a functioning governance framework covering validation, human oversight, monitoring, bias, explainability and accountability for clinical and commercial AI use?** | |
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| **Why it matters to the investor** | AI is frequently used to justify a valuation premium. If the "AI" is a thin layer on third-party models, trained on data the target cannot lawfully reuse, or lacks regulatory clearance, the premium is unsupported and the liabilities (patient harm, regulatory action, data misuse) transfer to the acquirer. Clinical decision support and diagnostics can also fall within medical device regulation, so regulatory status directly affects product continuity. |
| **Evidence / metrics needed** | • AI use-case register across diagnostics / imaging, triage and clinical decision support, revenue cycle and claims, drug discovery or pharmacovigilance, pricing and demand forecasting, field-force targeting, chatbots and patient engagement, and device-embedded software. For each: purpose, build versus buy, model owner, vendor, training data source, deployment status, affected users.<br>• Regulatory status of each clinical AI product, including whether it was assessed as software that is a medical device under the Medical Devices Rules, 2017 and any CDSCO licensing or classification; evidence of clinical validation on Indian patient populations; performance, drift and incident monitoring records.<br>• Governance artefacts: AI policy, risk classification, model validation protocol, human-in-the-loop design, override and escalation logs, bias testing, explainability approach, model change control, accountability owners, and board or committee oversight. Benchmark against ICMR's ethical guidelines for AI in biomedical research and healthcare (2023) and the national AI governance guidance in force at the time of diligence (confirm current status with counsel).<br>• Contractual and IP position: ownership of models and outputs, third-party / open-source licences, foundation-model vendor terms, indemnities.<br>• Hype test: measurable contribution to revenue, margin, clinical outcome or cost, versus the pre-AI baseline; customers that pay specifically for the AI capability; reproducibility of results by independent experts. |
| **Sector vs target** | Regulatory direction is sector-level; maturity, validation quality and data rights are target-specific. |
| **Signal it may reveal** | Distinguishes real competitive advantage from hype. An unapproved or unvalidated clinical AI product, or AI trained on data without a lawful basis, is a potential deal-breaker or a basis for price reduction and specific indemnity. |

| Q7 \| Area 5: Data governance, DPDP Act compliance and cybersecurity<br>**Does the target process patient and other personal data lawfully under the Digital Personal Data Protection Act, 2023 (and the rules made under it), with demonstrable consent, purpose limitation, security safeguards, breach readiness and a compliant approach to cross-border transfers, and how exposed is it to a material breach or enforcement?** | |
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| **Why it matters to the investor** | Healthcare is a high-volume processor of sensitive personal data, and a share acquisition brings historic non-compliance into the acquirer's group. The DPDP Act applies to digital personal data (and to non-digital data once digitised), carries significant monetary penalties set by the Data Protection Board, and does not treat health data as a separate "sensitive" tier, so health data is protected through the general obligations and through any Significant Data Fiduciary designation. Data rights also determine whether the data can be used for analytics and AI after closing. |
| **Evidence / metrics needed (DPDP-aligned)** | • Data map and role analysis: categories of personal data (patients, employees, doctors, insured members, research subjects, children), systems, locations, and whether the target is a Data Fiduciary or Processor for each flow.<br>• Lawful basis: consent artefacts that are free, specific, informed, unconditional and unambiguous, given by clear affirmative action, with itemised notice (Section 5) in the languages required; withdrawal mechanism; records showing reliance on the "certain legitimate uses" (Section 7, for example medical emergencies and health services in epidemic or threat to life, or voluntarily provided data) are accurate and not overstretched. Test legacy databases: how was consent or notice handled for data collected before commencement?<br>• Purpose limitation and retention: evidence that data is used only for the stated purpose and erased when the purpose is served, subject to retention duties under other laws (for example medical records and clinical trial rules).<br>• Children's data (Section 9): verifiable parental or guardian consent and absence of tracking, behavioural monitoring or targeted advertising directed at children, which is directly relevant to paediatric services and family health apps.<br>• Security safeguards and breach management (Section 8): encryption, access control and logging, vendor security, penetration test and audit results, incident history, and a tested process for notifying the Data Protection Board and each affected individual. Also check CERT-In directions (2022) on incident reporting and the applicable IT Act obligations in force during any transition period.<br>• Processor and vendor contracts (Section 8(2)): valid contracts with cloud, EMR, lab, telehealth, analytics and AI vendors, including sub-processors.<br>• Cross-border flows (Section 16): all transfers (including cloud hosting, offshore support, central-lab, clinical research and group reporting), and whether any transfer is to a country restricted by the Government; whether any sectoral law or contract imposes stricter localisation or transfer limits (the Act yields to stricter sectoral requirements).<br>• Data principal rights and grievance redress (access, correction, erasure, nomination, grievance officer) with response-time records.<br>• Significant Data Fiduciary exposure: scale and sensitivity of data may lead to designation, which adds a Data Protection Officer based in India, independent audit and data protection impact assessments. Check readiness.<br>• Deal-process compliance: the data room and transition should minimise personal data (anonymise or pseudonymise); do not assume that a share or asset deal automatically permits sharing or reusing personal data. Section 17 contains limited exemptions (including for certain court / tribunal-approved arrangements and for research or statistical purposes under conditions), which must be confirmed by counsel against the |

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| | actual transaction structure.<br>• DPDP Rules, 2025: test the target against the Rules as summarised in Section 5 (notice content, security safeguards, 72-hour breach reporting, log retention, children's data exemptions for healthcare, Significant Data Fiduciary duties, cross-border conditions), and against the phased commencement dates, which determine whether the target must be compliant at signing, at closing or during the first year of ownership. |
| **Sector vs target** | Law and enforcement approach are sector-level; the target's actual practices, historic breaches and data rights are target-specific. |
| **Signal it may reveal** | Potential deal-breaker if patient data was collected or transferred unlawfully, or if a material undisclosed breach exists. Otherwise a price, indemnity, escrow and 100-day remediation item. Strong governance can be a differentiator that supports platform value. |

| Q8 \| Area 6: Operational risk and service continuity<br>**Can the target sustain patient safety, clinical or product quality and uninterrupted supply and staffing at current and planned scale, and where are the single points of failure?** | |
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| **Why it matters to the investor** | Quality and continuity failures are the quickest route to value destruction: clinical incidents, product recalls, plant shutdowns, key-clinician departures or an ingredient shortage can interrupt revenue, trigger regulatory action and damage the brand. Growth plans that rely on adding capacity or sites multiply these risks. |
| **Evidence / metrics needed** | • Clinical quality indicators: infection rates, sentinel events, mortality and readmission measures, patient complaints, NABH or equivalent audit findings, medical negligence claims and insurer settlements. Product quality: batch failure and complaint rates, CAPA closure, recall history, pharmacovigilance and materiovigilance reports.<br>• Supply chain: dependency on imported active ingredients, components and consumables by country and supplier, single-source items, inventory cover, cold-chain integrity, distributor concentration and channel stock.<br>• Workforce: doctor, nurse and technician attrition and vacancy, reliance on a few key clinicians or promoters, consultant contract terms and non-compete enforceability, wage inflation.<br>• Business continuity and disaster recovery plans and test results; site concentration; insurance cover for professional liability, product liability and cyber.<br>• Promoter and key person dependency and succession plan. |
| **Sector vs target** | Sector-wide pressures (talent scarcity, import reliance) matter, but exposure is target-specific. |
| **Signal it may reveal** | Potential deal-breaker if there are unresolved patient-safety or product-quality failures; otherwise integration priorities and working-capital or retention-based deal protections (key-person retention, earn-out tied to quality). |

| Q9 \| Area 7: Financial quality, payer exposure and valuation<br>**How high-quality, recurring and cash-convertible is the target's revenue given its payer mix, contract terms and reimbursement exposure, and what valuation does the verified earnings base support?** | |
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| **Why it matters to the investor** | Reported EBITDA can overstate sustainable earnings when revenue relies on a few payers, tenders, distributors or promoter-related parties, when receivables stretch, or when growth depends on discounts or one-off items. In India, insurer / TPA and government scheme payers can delay or reject claims and reset rates, directly affecting cash conversion |

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| | and margins. |
| **Evidence / metrics needed** | • Quality of earnings review: revenue recognition, normalisation adjustments, related-party transactions, capitalised costs, off-balance-sheet liabilities, contingent liabilities, and tax position (including GST and transfer pricing disputes).<br>• Payer and customer mix: cash, insurer / TPA, corporate, CGHS, PM-JAY and state schemes, institutional and tender customers, distributors, export customers; concentration; contract tenor, renewal and termination rights, change-of-control clauses, rate-revision history; claim rejection and deduction rates; receivable ageing and days.<br>• Unit economics: by facility, product, therapeutic area or customer cohort; contribution margin, capex intensity and return on capital employed; cash conversion.<br>• Valuation triangulation using verified numbers: DCF with explicit scenarios (see Q10), peer multiples and precedents (Q2), with disclosed adjustments for regulatory, data and quality risks identified in Q3, Q6, Q7 and Q8.<br>• Currency, financing and repatriation considerations for a foreign acquirer. |
| **Sector vs target** | Target-specific, using sector reimbursement and valuation trends as context. |
| **Signal it may reveal** | The main determinant of price. Weak revenue quality or hidden liabilities are common deal-breakers or grounds for earn-out, escrow, or locked-box protections. |

| **Q10 \| Area 8: Emerging threats, future trends and scenarios**<br>**Which regulatory shifts, technology developments and market disruptors could materially change the target's economics over the holding period, and does the investment case remain acceptable under defined downside scenarios?** | |
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| **Why it matters to the investor** | A transaction priced on today's regulation and competitive structure can underperform if rules or business models change. Scenario analysis tests resilience and clarifies what must be true to proceed, and what protections or price are needed if it does not hold. |
| **Evidence / metrics needed** | • Regulatory pipeline: further price or margin regulation; medical device and software regulation; e-pharmacy and telemedicine rules; AI-specific governance; DPDP enforcement practice and any designations or guidance by the Data Protection Board; changes to foreign investment policy; scheme reimbursement rate changes. Track official consultation papers and notifications.<br>• Disruptors: new entrants and platform models (digital pharmacy, diagnostics aggregators, insurtech, integrated payer-provider models), generative AI and clinical automation, medical tourism and trade policy changes affecting exports, supply shocks in imported inputs, and cyber threats targeting health data.<br>• Scenario set defined by the deal team (base, regulatory-downside, quality-incident, data-breach, competitive-price-war), each with explicit assumptions and the effect on revenue, margin, capex and exit multiple. Do not use a scenario unless its drivers can be tied to cited evidence.<br>• Expert interviews (regulatory counsel, clinicians, customers, former executives) to test assumptions. |
| **Sector vs target** | Sector-level threats; the target's sensitivity to each is target-specific. |
| **Signal it may reveal** | Threats that can be priced or structured around versus those that make the deal unacceptable. Also reveals upside options, such as expanding into adjacent segments, if regulation or technology moves favourably. |

## 4. Validation gates for the investment decision

Each gate links a question to a specific approval test. The committee should treat a failed deal-breaker gate as a stop or restructure decision, and a failed price gate as a mandate to renegotiate or add protection.

| Gate | What must be true | Questions | If it fails |
|---|---|---|---|
| **G1 Licences and enforcement** | All material licences valid, scoped to current activities, and continuing or re-grantable on change of control; no unresolved serious enforcement action. | Q3 | Deal-breaker |
| **G2 Transaction path** | Foreign investment route, CCI and other approvals confirmed by counsel; structure preserves the control the investment case needs. | Q4 | Deal-breaker / structure |
| **G3 Verified earnings** | Quality of earnings confirms the EBITDA and cash conversion basis; no material undisclosed liabilities or concentrated payer risk beyond tolerance. | Q9 | Price / deal-breaker |
| **G4 Data lawfulness** | Personal data collected, used and transferred on a defensible DPDP basis; no material unremediated breach; vendor and cross-border flows documented. | Q7 | Deal-breaker / indemnity |
| **G5 Clinical and AI safety** | Clinical AI products have regulatory status and validation evidence; governance operating in practice; AI claims verified. | Q6 | Price / indemnity |
| **G6 Quality and continuity** | No unresolved systemic patient-safety or product-quality failure; supply and key-person risks mitigated. | Q8 | Deal-breaker / protections |
| **G7 Growth thesis** | Growth is attributable to structural drivers in segments the target can capture; valuation consistent with precedents and peers. | Q1, Q2 | Price |
| **G8 Integration and resilience** | Integration cost and timeline acceptable; downside scenarios remain within the acquirer's return and risk tolerance. | Q5, Q10 | Price / plan |

### Sequencing the work

- **Weeks 1-2, red-flag screen:** licence register and transferability (Q3), approval path (Q4), data map and breach history (Q7), recall or patient-safety incidents (Q8). These can end or reshape the deal cheaply.
- **Weeks 2-6, core diligence:** quality of earnings and payer analysis (Q9), market and competitor work (Q1, Q2), technology and AI diligence (Q5, Q6), full DPDP compliance review (Q7).
- **Final stage, structuring:** scenario valuation (Q10), conditions precedent, indemnities, escrow or earn-outs, and a 100-day remediation and integration plan.

*The timings above are indicative and should be adjusted to the transaction timetable.*

## 5. DPDP Rules, 2025: diligence alignment

**Verification status.** This section reflects our understanding of the DPDP Rules, 2025 as notified in November 2025. We could not retrieve the official text in this session, so every row below must be checked against the Gazette

notification by Indian counsel before it is relied on. Rule numbers are given only where we are confident; otherwise the topic is named.

| Topic | Our understanding of the Rules | What to test in the target |
|---|---|---|
| **Phased commencement** | Board-establishment provisions took effect on notification. Consent-manager provisions apply after about 12 months and the main fiduciary obligations after about 18 months, which points to roughly mid-2027. Confirm the dates and any announced changes to the timetable. | Until the main obligations commence, the target remains subject to existing IT Act / SPDI Rules duties. A deal closing in 2026-27 will inherit the remediation workload. Ask for the target's DPDP readiness plan, budget and owner, and price the gap. |
| **Notice** | Notice must be standalone, clear and plain, itemise the data and the specific purposes, and explain how to withdraw consent and exercise rights and how to complain to the Board. | Sample patient, employee, doctor and app notices; language coverage; whether purposes are specific rather than bundled. |
| **Security safeguards** | Minimum safeguards include encryption, obfuscation or masking, access controls, logging and monitoring, backups and continuity measures, and contractual safeguards for processors. | Technical evidence, not policy: penetration tests, access reviews, log coverage, restore tests, processor audit rights. |
| **Breach reporting** | Notify affected individuals without delay, notify the Board without delay, and provide a detailed report to the Board within a stated period (understood to be 72 hours). | Incident response plan mapped to this sequence; breach register for the last several years; tabletop exercise results. Align with the CERT-In reporting duty, which is separate. |
| **Log and data retention** | Logs and personal data processed must be retained for a minimum period (understood to be one year for logs), alongside erasure obligations when the purpose ends. | Reconcile with medical-record retention and clinical-trial rules; check whether erasure and retention are automated and which law the target follows when they conflict. |
| **Children's data** | Verifiable parental consent is required, with exemptions for certain classes. We understand that clinical establishments and healthcare professionals are exempted to the extent needed to provide health services to a child. | Confirm the exemption scope covers each of the target's uses. It may not cover marketing, wellness apps or analytics, which remain high risk. |
| **Consent managers** | Consent managers must be registered Indian entities meeting prescribed conditions; interaction with ABDM consent architecture should be reviewed. | Whether the target plans to integrate, and how its ABDM consent flows relate to DPDP consent. |

| Topic | Our understanding of the Rules | What to test in the target |
|---|---|---|
| **Significant Data Fiduciary** | Additional duties may include annual data protection impact assessment and audit, due diligence on algorithmic software used for processing, and restrictions on transfers of specified data as the Government prescribes. | Likelihood of designation given volume and sensitivity; readiness of DPO, DPIA and audit; whether AI models fall under the algorithmic due diligence duty. |
| **Cross-border transfers** | Transfers are permitted subject to requirements the Government may prescribe, and subject to stricter sectoral laws. | Map every offshore flow; identify contracts and hosting that would need to change if restrictions follow; group-level data sharing after closing. |
| **Research and statistical use** | Exemptions exist for research, archiving and statistical purposes if prescribed standards are met. | Whether the target's model training and analytics qualify, or rely on consent and a lawful basis that may not exist. |

### Where this changes the decision

- Price: remediation cost and a potential penalty exposure after commencement should be reflected in the valuation or through a specific indemnity, with the amount set by counsel from the Act's penalty schedule.
- Structure: add warranties on pre-closing compliance, conditions precedent for priority fixes, and a covenant to implement a compliance programme within a defined period after closing.
- AI value: if training data lacks a lawful basis, discount the AI asset, or require retraining or re-consent as a condition of closing.

## 6. Our own data governance on this engagement

**Status.** You asked that the work follow PwC's internal data governance. We do not have that policy in this session, and we have not assumed what it says. The controls below are general good practice for a diligence workstream and must be reconciled with the policy once we have it. Please share the policy or name the owner, and we will map each point to its clauses.

- No personal data in the sector scan or in the question set. Use anonymised or aggregated data in the data room wherever the question can be answered that way.
- Classify target documents by sensitivity and restrict access to named deal team members; log access and keep an audit trail.
- Store target information only in approved repositories and retain it only for the engagement period, with documented return or destruction after closing or termination.
- Do not enter target confidential or personal data into any AI tool or external service unless it is approved under the policy; record any AI-assisted analysis and have a person review it.
- Cite sources for every external fact, mark unverified items, and escalate any suspected personal data incident immediately through the firm's incident process.

## 7. Implications of evolving AI and healthcare regulation for M&A in India

- Regulatory status will increasingly be part of asset value. Clinical software and AI that falls within medical device regulation needs clearance, and the absence of it can be a barrier to continued sale after closing.

- DPDP Act enforcement will make data provenance a core diligence item. Data collected without a valid notice and consent basis may be neither lawfully transferable nor reusable for model training, which affects the value of data assets and AI.
- Deal protections should evolve accordingly: specific data-protection and AI warranties, indemnities for pre-closing non-compliance, escrow, conditions precedent for remediation, and post-closing governance covenants.
- Policy remains in motion (DPDP rules and phasing, AI governance guidance, device and price regulation, foreign investment policy). The transaction documents should allocate change-in-law risk explicitly.

## 8. Limitations

This scan frames questions and evidence requirements; it does not answer them. It is not legal, tax or investment advice. Legal conclusions on licensing, foreign investment, competition and the DPDP Act must be confirmed by Indian counsel. The first deliverable after target confirmation should be an evidence request list built from Section 3.